

Anesthesia Record

Patient Demographics

Name	Patient ID	SSN	Gender Identity	Birth Date
██████████ R	██████████	██████████	██████████	██████████
Address	Phone	Email		
████████████████████	██████████████████	██████████████████		
Reg Status	PCP	Date Last Verified	Next Review Date	
██████████	██████████████████	██████████	██████████	

Subscriber Details

				Hospital Account
CVG	Subscriber Name/Sex/Relation	Subscriber DOB	Subscriber	Subscriber Emp/Emp
██████████	██████████████████	██████████	██████████████████	

Guarantor Information

Name:	██████████	SSN:	██████████
Address:	██████████████████		
City:	██████████	State:	██████████
		Zip:	██████████
		Phone:	██████████
Employer:			
Address:			
City:	██████████	State:	██████████
		Zip:	██████████
		Phone:	██████████
Guar DOB:	██████████		

Anesthesia Summary -

Height: 1.626 m (5' 4") (06/21/23)
Weight: 113.4 kg (250 lb) (06/21/23)
BMI: ██████████
NPO Status: 2000
Allergies: No Known Allergies

General Information

Date: 6/21/2023	Case classification: Scheduled	Status: Unposted
Location: ██████████	Room: OR 01	Service: Orthopedics
Patient class: Hospital Ambulatory		
Surgery		

Procedure Info

Panel 1				
Surgeon		Role		Service
[REDACTED]		Primary		Orthopedics
Procedure: LEFT KNEE ARTHROSCOPY, PARTIAL MEDIAL AND LATERAL MENISCECTOMY [29882 (CPT®)]				
Laterality	Wound Class	Incision Closure	Anesthesia	Op Region
Left	Clean		General	

Diagnosis (required)

Pre-op diagnosis: Tear of medial meniscus of left knee, unspecified tear type, unspecified whether old or current tear, initial encounter [S83.242A]Tear of lateral meniscus of left knee, unspecified tear type, unspecified whether old or current tear, initial encounter [, S83.282A]
Post-op diagnosis: Tear of medial meniscus of left knee, unspecified tear type, unspecified whether old or current tear, initial encounter [S83.242A]Tear of lateral meniscus of left knee, unspecified tear type, unspecified whether old or current tear, initial encounter [, S83.282A]

Pre Vitals

Current as

Pre Vitals (continued)

Current as

BP: 115/64
Resp: 16
Temp: 97.3 °F (36.3 °C)
Height: 1.626 m (5' 4") (06/21/23)
BMI:
Last edited 06/21/23 0811 by

Pulse: 70
SpO2: 98
Weight: 113.4 kg (250 lb) (06/21/23)

Problem List

Current as

Episode of recurrent major depressive disorder (HCC)
Hydronephrosis
Kidney stone

Nephrolithiasis
Obstructed umbilical hernia
Type 2 diabetes mellitus with hyperglycemia, without long-term current use of insulin (HCC)
Ventral hernia

Essential hypertension
Intractable pain
Migraine without aura and without status migrainosus, not intractable
Obesity, unspecified
Partial small bowel obstruction (HCC)
Umbilical hernia, incarcerated

Anesthesia History

No specialty history recorded

Other Medical History

Reactive depression (situational)
Migraine
Psoriasis
Diabetes mellitus (HCC)
Tonsillar and adenoid hypertrophy
Sleep apnea

Kidney calculi
Umbilical hernia without obstruction and without gangrene
Depression
HTN (hypertension)
Former cigarette smoker
CPAP (continuous positive airway pressure) dependence

Surgical History

CESAREAN SECTION
LAPAROSCOPIC ROBOTIC ASSISTED UMBILICAL HERNIORRHAPHY

CESAREAN SECTION W/ TUBALIGATION
CYSTOSCOPY WITH URETEROSCOPY WITH LASER LITHOTRIPSY WITH INSERTION STENT

Social History**Tobacco History**

Smoking Status	Quit Date	Smoking Frequency	Smoking Tobacco Type
Former	6/1/2005	0.50 packs/day for 8.00 years (4.00 pk-yrs)	Cigarettes quit in 6/1/2005
Smokeless Tobacco Use			
Never			
Tobacco Comments			
quit 2006, pack lasted 1 week			

Alcohol History

Alcohol Use Status	Comment
Yes	socially/ rarely

Drug Use

Drug Use Status	Comment
Yes	smokes marijuana daily

All Component Based Labs (Last result in the past 60 days)

All Component Based Labs (continued) (Last result in the past 60 days)

Hgb	Hct	WBC	Platelet	PT	INR	APTT	Preg Qual	Pregnancy Qualitative Urine	ABO/Rh
04/24/23	04/24/23	04/24/23	04/24/23						
1222	1222	1222	1222						
11.6	36.7	9.25	378						

All Component Based Labs (Last result in the past 60 days)

Sodium Lvl	Potassium Lvl	Chloride	CO2	BUN	Creatinine	Mean Plasma Glucose	AST	ALT
04/24/23	04/24/23	04/24/23	04/24/23	04/24/23	04/24/23	04/24/23	04/24/23	04/24/23
1222	1222	1222	1222	1222	1222	1222	1222	1222
138	3.9	102	23	8 ▼	0.5	167 ▲	17	12 ▼

All Component Based Labs (Last result in the past 60 days)

ABO/Rh	BUN	Creatinine	Mean Plasma Glucose
	04/24/23	04/24/23	04/24/23
	1222	1222	1222
	8 ▼	0.5	167 ▲

All Component Based Labs (Last result in the past 60 days)

AST	ALT	Magnesium
04/24/23	04/24/23	
1222	1222	
17	12 ▼	

Pre Signoff

Cleared for Anesthesia marked on 06/21/23 at 0844 by [REDACTED]

Prescription Medications

Within last 14 days from 06/21/23

	Last Taken	Last Updated
escitalopram (LEXAPRO) 20 MG tablet	6/20/2023	06/21/23 0810
ibuprofen (MOTRIN) 800 MG tablet	6/14/2023	06/21/23 0810
Liraglutide -Weight Management (SAXENDA) 18 MG/3ML PEN injection	6/20/2023	06/21/23 0810
lisinopril (PRINIVIL) 20 MG tablet	6/21/2023	06/21/23 0810
MetFORMIN HCl 750 MG SR tablets	6/20/2023	06/21/23 0810
ondansetron, disintegrating, (ZOFTRAN ODT) 4 MG dispersible tablet	Taking-	06/15/23 0847
tamsulosin (FLOMAX) 0.4 MG capsule	6/20/2023	06/21/23 0810
topiramate (TOPAMAX) 50 MG tablet	6/20/2023	06/21/23 0810
triamcinolone acetonide (KENALOG) 0.5 % topical cream	Taking-	06/15/23 0847
Liraglutide -Weight Management (SAXENDA) 18 MG/3ML PEN-injection (Discontinued)	Taking-	06/15/23 0847
Liraglutide -Weight Management (SAXENDA) 18 MG/3ML PEN-injection (Discontinued)		
triamcinolone acetonide (KENALOG) 0.025 % topical ointment (Discontinued)	Not Taking	06/15/23 0847

Preop Evaluation Note

Last edited 06/21/23 0844 by [REDACTED]
Creation Time 06/21/23 0842

Preop Evaluation Note (continued)

Status: Signed

Anesthesia Plan

ASA Score: 3

Mallampati: II

Possible difficult intubation? No

Anesthesia plan: General

Anesthetic plan and risks discussed with Patient.

Special Instructions: Knee pain, limits activity, can walk one block without SOB or CP.

Patient has lip ring in place, explained there could be trauma with airway manipulation, patient understands and consents to proceed.

Patient had recent full mouth dental extraction 6/6/23, gums appear to be healed, there may be some incidental trauma with LMA, patient understands and consents to proceed.

Facility Administered Medications

Taken on 06/21/23

acetaminophen (TYLENOL) tablet 650 mg

famotidine (PEPCID) tablet 20 mg

ibuprofen (MOTRIN) tablet 600 mg

Responsible Staff

06/21/23

Name	Role	Begin	End
[REDACTED]	ANESTH	0908	1006
	CRNA	0908	1006

Events

Date	Time	Event
6/21/2023	[REDACTED]	Cleared for Anesthesia
		Anesthesia Start
		Induction
		LMA Applied
		Emergence
		Extubation
		Stop Data Collection
		Anesthesia Stop

Block Note

No Anesthesia note exists

Device History

Device	Currently Attached?	Attached	Detached
MSC_OR_4 [170000004]	No	6/21/2023 9:08 [REDACTED] ANES START DATA COLLECTION	[REDACTED] STOP DATA COLLECTION

Checklist

	6/21/2023 0847
Room Temp Increased:	No
Re-evaluate Prior to Induct.:	Yes

Checklist (continued)

6/21/2023 0847	
Medcart Appropriately Stocked:	Yes
Sufficient Gas Supply:	Yes

Case Tracking Events

Event	Time In
In Procedure Room	0908
Procedure Started	0929
Procedure Completed	0957
Out of Procedure Room	1003

Lines, Drains, and Airways

Type	Details	Placement	Removal
Peripheral IV	06/21/23; 0820; Normal Saline; Right; Hand; 20 GA; 1; Tolerated Well; 06/21/23; 1114		
ETT Group	06/21/23; 0914; LMA; Cuffed; Oral; 4; Auscultation, EXCO2; 1; No; smooth induction, atraumatic, easy LMA insertion; 06/21/23; 1000		

Positioning

6/21/2023 0908	
Position:	Supine
Checklist:	Eyes Taped;PP Checked;PP Padded;Arms Padded;Arms < 90;Neck Stable

Attestation Information

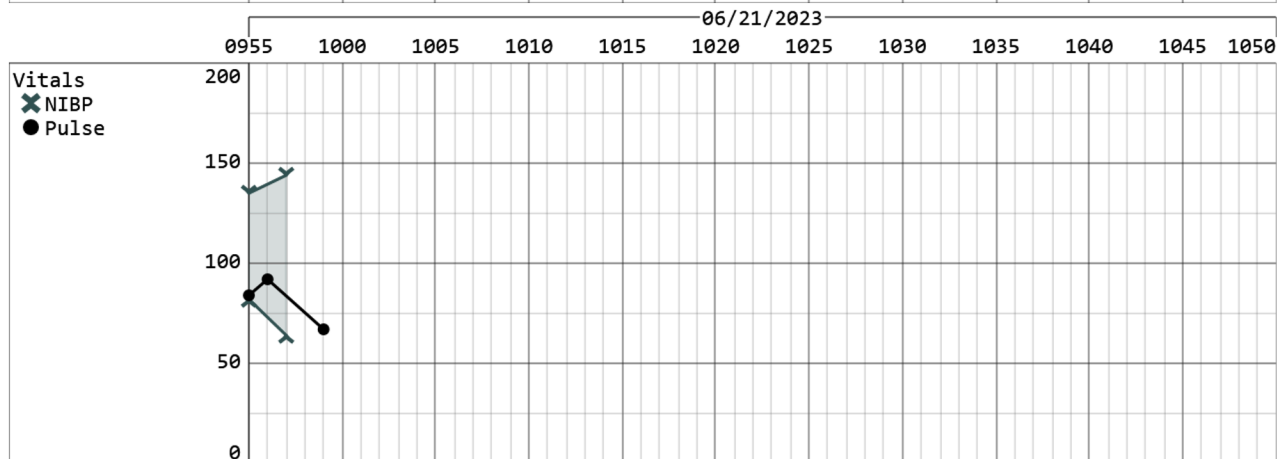
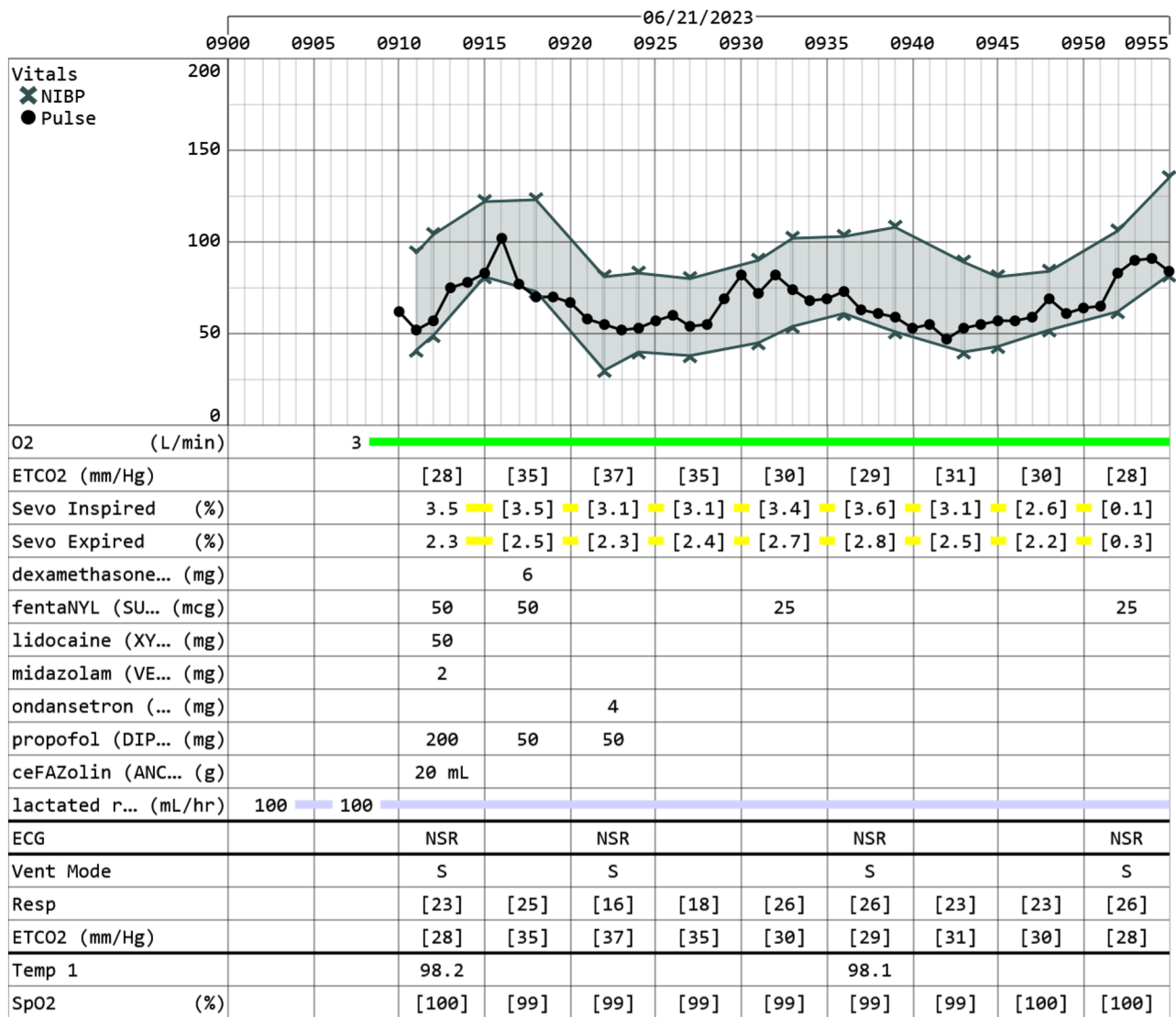
Staff Name	Date	Time	Type
	06/21/23	0908	MDA Available Throughout Case
	procedure, continuously monitored the course of anesthetic care, and was		
	06/21/23	0910	Present at Induction
	induction of anesthesia. By signing, I attest that I have identified and re-		
	re the induction of anesthesia and I ■ satisfied that my anesthetic plan is		
	procedure. The first vital signs recorded are pre-induction. 		
	06/21/23	1006	Present at Emergence
	management of this patient during emergence from general		

Assessments

	6/21/2023 0910	6/21/2023 0924	6/21/2023 0935	6/21/2023 0950
L Breath Sounds:	Clear	Clear	Clear	Clear
R Breath Sounds:	Clear	Clear	Clear	Clear
Eyes Free of Pressure:	Yes	Yes	Yes	Yes

Block Note

No Anesthesia note exists



O2 (L/min)	3	0										
ETCO2 (mm/Hg)	[31]	0										
Sevo Inspired (%)	[0.1]	0										
Sevo Expired (%)	[0.2]	0										
dexamethasone... (mg)											6 mg	
fentaNYL (SU... (mcg)											150 mcg	
lidocaine (XY... (mg)											50 mg	
midazolam (VE... (mg)											2 mg	
ondansetron (... (mg)											■ mg	
propofol (DIP... (mg)											300 mg	
ceFAZolin (ANC... (g)											2 g	
lactated r... (mL/hr)	100											
ECG												
Vent Mode												
Resp	[31]											
ETCO2 (mm/Hg)	[31]	0										
Temp 1												
SpO2 (%)	[88]											

PACU Handoff Notes

Last edited 06/21/23 1007 by [REDACTED]

Creation Time 06/21/23 1007

Status: Signed

Post Anesthesia Assessment

Patient: [REDACTED]

Procedure(s) Performed: Procedure(s):
LEFT KNEE ARTHROSCOPY

Post assessment: no apparent anesthetic complications, tolerated procedure well and no evidence of recall

Post vital signs: BP - 130/78, HR - 80, TEMP - 97.3 °F (36.3 °C), RR - Resp: (!) 22, PULSE OX - 99 %

Level of consciousness: awake and alert

Report given to RN

All Post Notes

Last edited 06/21/23 1303 by [REDACTED]

Creation Time 06/21/23 1303

Status: Signed

Discharge Evaluation Note

Vitals in patient's normal range: **Yes**

Respiratory status is acceptable: **Yes**

Airway patency maintained: **Yes**

Cardiovascular status is stable: **Yes**

Hydration status is balanced: **Yes**

Patient participated in Post Anesthesia Evaluation: **Yes**

Pain is controlled: **Yes**
Nausea and vomiting controlled: **Yes**

Patient Demographics

Name	Patient ID	SSN	Gender Identity	Birth Date
██████████ R	██████████	██████████	██████████	██████████
Address	Phone	Email		
██████████	██████████	██████████		
Reg Status	PCP	Date Last Verified	Next Review Date	
██████████	██████████	██████████	██████████	

CVG	Subscriber Name/Sex/Relation	Subscriber DOB	Subscriber	Subscriber Emp/Emp
[REDACTED]	[REDACTED]	[REDACTED]	[REDACTED]	[REDACTED]

Name:				SSN:	
Address:					
City:		State:		Zip:	
				Phone:	
Employer:					
Address:					
City:		State:		Zip:	
Guar DOB:				Phone:	

Date: 6/21/2023	Case classification: Scheduled	Status: Unposted
Location: [REDACTED]	Room: OR 01	Service: Orthopedics
Patient class: Hospital Ambulatory Surgery		

Date: 06/21/23
Surgeon: [REDACTED]
Responsible Provider: [REDACTED]
Location: M [REDACTED]

Procedure [Code]
LEFT KNEE ARTHROSCOPY, PARTIAL MEDIAL AND LATERAL MENISCECTOMY (Left) [29882 CPT (R)]

Diagnosis [Codes]
Tear of medial meniscus of left knee, unspecified tear type, unspecified whether old or current tear, initial encounter [S83.242A]
Tear of lateral meniscus of left knee, unspecified tear type, unspecified whether old or current tear, initial encounter [S83.282A]

Preoperative diagnosis

Case Summary (continued)

Tear of medial meniscus of left knee, unspecified tear type, unspecified whether old or current tear, initial encounter [S83.242A]Tear of lateral meniscus of left knee, unspecified tear type, unspecified whether old or current tear, initial encounter [S83.282A]

Responsible Staff

06/21/23

Name	Role	Begin	End
	ANESTH	0908	1006
	CRNA	0908	1006

Preop Evaluation Note

Last edited 06/21/23 0844 by
Creation Time 06/21/23 0842
Status: Signed

Anesthesia Plan

ASA Score: 3

Mallampati: II

Possible difficult intubation? No

Anesthesia plan: General

Anesthetic plan and risks discussed with Patient.

Special Instructions: Knee pain, limits activity, can walk one block without SOB or CP.

Patient has lip ring in place, explained there could be trauma with airway manipulation, patient understands and consents to proceed.

Patient had recent full mouth dental extraction 6/6/23, gums appear to be healed, there may be some incidental trauma with LMA, patient understands and consents to proceed.

Anesthesia Plan

	6/9/2023 1251
Teeth Assessment:	Missing

Events

Date	Time	Event
6/21/2023	0844	Cleared for Anesthesia
	0908	Anesthesia Start
	0910	Induction
	0914	LMA Applied
	0954	Emergence
	1000	Extubation
	1002	Stop Data Collection
	1006	Anesthesia Stop

Case Tracking Events

Event	Time In
In Procedure Room	0908
Procedure Started	0929
Procedure Completed	0957
Out of Procedure Room	1003

Lines, Drains, and Airways

Type	Details	Placement	Removal
Peripheral IV	06/21/23; 0820; Normal Saline; Right; Hand; 20 GA; 1; Tolerated Well; 06/21/23; 1114		
ETT Group	06/21/23; 0914; LMA; Cuffed; Oral; 4; Auscultation, EXCO2; 1; No; smooth induction, atraumatic, easy LMA insertion; 06/21/23; 1000		

Checklist

	6/21/2023 0847
Room Temp Increased:	No
Re-evaluate Prior to Induct.:	Yes
Medcart Appropriately Stocked:	Yes
Sufficient Gas Supply:	Yes

Positioning

	6/21/2023 0908
Position:	Supine
Checklist:	Eyes Taped; PP Checked; PP Padded; Arms Padded; Arms < 90; Neck Stable

Block Note

No Anesthesia note exists

Final Anesthesia Type

Type: General

PACU Handoff Notes

Last edited 06/21/23 1007 by [REDACTED]
Creation Time 06/21/23 1007
Status: Signed

Post Anesthesia Assessment

Patient: [REDACTED]

Procedure(s) Performed: Procedure(s):
LEFT KNEE ARTHROSCOPY

Post assessment: no apparent anesthetic complications, tolerated procedure well and no evidence of recall

Post vital signs: BP - 130/78, HR - 80, TEMP - 97.3 °F (36.3 °C), RR - Resp: (!) 22, PULSE OX - 99 %

Level of consciousness: awake and alert

Report given to RN

All Postprocedure Notes

Last edited 06/21/23 1303 by [REDACTED]

All Postprocedure Notes (continued)

Creation Time 06/21/23 1303

Status: Signed

Discharge Evaluation Note

Vitals in patient's normal range: **Yes**

Respiratory status is acceptable: **Yes**

Airway patency maintained: **Yes**

Cardiovascular status is stable: **Yes**

Hydration status is balanced: **Yes**

Patient participated in Post Anesthesia Evaluation: **Yes**

Pain is controlled: **Yes**

Nausea and vomiting controlled: **Yes**

Comments: **No apparent anesthetic complications.**

Electronically signed by

Timeout Information

Timeout Type -: Preprocedure

Verification Staff

Performed Instant

6/21/2023 9:27

Surgeon:

MICHE

Correct patient?: Yes

Correct site?: Yes

Correct position?: Yes

Correct procedure?: Yes

Are the H&P and consent complete and verified, and in agreement with the scheduled procedure?: **Yes**

Radiology studies available?: **Yes**

Anesthesia states that monitoring is on patient?: **Yes**

Monitoring equipment on patient?: **Yes**

Irrigations / Medications available?: **Yes**

Irrigations / Medications available?: **Yes**

Confirmed correct radiology images for patient?: **Yes**

Comments: Lori Dusenbery observer

Intra-procedure Meds

dexamethasone (DECADRON) ■ mg/mL injection (mg)

Date/Time	Rate/Dose/Volume	Action
06/21/23 0919	6 mg	Given

fentaNYL (SUBLIMAZE) 50 mcg/ml injection (mcg)

Date/Time	Rate/Dose/Volume	Action
06/21/23 0911	50 mcg	Given
0919	50 mcg	Given
0933	25 mcg	Given
0953	25 mcg	Given

lidocaine (XYLOCAINE) 2 % injection (mg)

Date/Time	Rate/Dose/Volume	Action
06/21/23 0912	50 mg	Given

midazolam (VERSED) 2 mg/2 mL injection (mg)

Date/Time	Rate/Dose/Volume	Action
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Intra-procedure Meds (continued)

06/21/23 0911	2 mg	Given
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ondansetron (ZOFTRAN) 2 mg/mL injection (mg)

Date/Time	Rate/Dose/Volume	Action
06/21/23 0924	4 mg	Given

propofol (DIPRIVAN) 200mg/20mL injection (mg)

Date/Time	Rate/Dose/Volume	Action
06/21/23 0912	200 mg	Given
0919	50 mg	Given
0924	50 mg	Given

ceFAZolin (ANCEF) 2,000 mg in sterile water 20 mL piggyback (g) Dosing weight: 113.4

Date/Time	Rate/Dose/Volume	Action
06/21/23 0913	2 g - 20 mL	New Bag

lactated ringers infusion (mL/hr) Dosing weight: 113.4

Date/Time	Rate/Dose/Volume	Action
06/21/23 0903	100 mL/hr	New Bag
0908	100 mL/hr	Continued by Anesthesia

MR Coding Query

Patient's encounter data is missing

Final Anesthesia Type

Type: General